

Disorders of Impulse Control Not Elsewhere Classified

This is a residual diagnostic class for disorders of impulse control that are not classified in other categories, e.g., as a Substance Use Disorder or Paraphilia.

The essential features of disorders of impulse control are:

1. Failure to resist an impulse, drive, or temptation to perform some act that is harmful to the individual or others. There may or may not be conscious resistance to the impulse. The act may or may not be premeditated or planned.
2. An increasing sense of tension before committing the act.
3. An experience of either pleasure, gratification, or release at the time of committing the act. The act is ego-syntonic in that it is consonant with the immediate conscious wish of the individual. Immediately following the act there may or may not be genuine regret, self-reproach, or guilt.

This class contains five specific categories: Pathological Gambling, Kleptomania, Pyromania, Intermittent Explosive Disorder, and Isolated Explosive Disorder. Finally, there is a residual category, Atypical Impulse Control Disorder.

312.31 Pathological Gambling

The essential features are a chronic and progressive failure to resist impulses to gamble and gambling behavior that compromises, disrupts, or damages personal, family, or vocational pursuits. The gambling preoccupation, urge, and activity increase during periods of stress. Problems that arise as a result of the gambling lead to an intensification of the gambling behavior. Characteristic problems include loss of work due to absences in order to gamble, defaulting on debts and other financial responsibilities, disrupted family relationships, borrowing money from illegal sources, forgery, fraud, embezzlement, and income tax evasion.

Commonly these individuals have the attitude that money causes and is also the solution to all their problems. As the gambling increases, the individual is usually forced to lie in order to obtain money and to continue gambling, but hides the extent of the gambling. There is no serious attempt to budget or save money. When borrowing resources are strained, antisocial behavior in order to obtain money for more gambling is likely. Any criminal behavior—e.g., forgery, embezzlement, or fraud—is typically nonviolent. There is a conscious intent to return or repay the money.

Associated features. These individuals most often are overconfident, somewhat abrasive, very energetic, and “big spenders”; but there are times when they show obvious signs of personal stress, anxiety, and depression.

Age at onset and course. The disorder usually begins in adolescence and waxes and wanes, tending to be chronic.

Impairment. The disorder is extremely incapacitating and results in failure to maintain financial solvency or provide basic support for oneself or one's family. The individual may become alienated from family and acquaintances and may lose what he or she has accomplished or attained in life.

Complications. Suicide attempts, association with fringe and illegal groups, and arrest for nonviolent crimes that may lead to imprisonment are among the possible complications.

Predisposing factors. These may include: loss of parent by death, separation, divorce, or desertion before the child is 15 years of age; inappropriate parental discipline (absence, inconsistency, or harshness); exposure to gambling activities as an adolescent; a high family value on material and financial symbols; and lack of family emphasis on saving, planning, and budgeting.

Prevalence. No information.

Sex ratio. The disorder is apparently more common among males than females.

Familial pattern. Pathological Gambling and Alcoholism are more common in the fathers of males and in the mothers of females with the disorder than in the general population.

Differential diagnosis. In **social gambling**, gambling with friends is engaged in mainly on special occasions and with predetermined acceptable losses.

During a **manic or hypomanic episode** loss of judgment and excessive gambling may follow the onset of the mood disturbance. When manic-like mood changes occur in Pathological Gambling they typically follow winning.

Problems with gambling are often associated with **Antisocial Personality Disorder** and in Pathological Gambling antisocial behavior is frequent. However, in Pathological Gambling any antisocial behavior that occurs is out of desperation to obtain money to gamble when money is no longer available and legal resources have been exhausted. Criminal behavior is rare when the individual has money. Also, unlike the individual with Antisocial Personality Disorder, the individual with Pathological Gambling usually has a good work history until it is disrupted because of the gambling.

Diagnostic criteria for Pathological Gambling

A. The individual is chronically and progressively unable to resist impulses to gamble.

B. Gambling compromises, disrupts, or damages family, personal, and vocational pursuits, as indicated by at least three of the following:

- (1) arrest for forgery, fraud, embezzlement, or income tax evasion due to attempts to obtain money for gambling
- (2) default on debts or other financial responsibilities
- (3) disrupted family or spouse relationship due to gambling
- (4) borrowing of money from illegal sources (loan sharks)
- (5) inability to account for loss of money or to produce evidence of winning money, if this is claimed
- (6) loss of work due to absenteeism in order to pursue gambling activity
- (7) necessity for another person to provide money to relieve a desperate financial situation

C. The gambling is not due to Antisocial Personality Disorder.

312.32 Kleptomania

The essential feature is a recurrent failure to resist impulses to steal objects that are not for immediate use or their monetary value: the objects taken are either given away, returned surreptitiously, or kept and hidden. Almost invariably the individual has enough money to pay for the stolen objects. The individual experiences an increasing sense of tension before committing the act and intense gratification while committing it. Although the theft does not occur when immediate arrest is probable (e.g., in full view of a policeman), it is not preplanned, and the chances of apprehension are not fully taken into account. The stealing is done without long-term planning and without assistance from, or collaboration with, others.

The diagnosis is not made if the stealing is due to Conduct Disorder or Antisocial Personality Disorder.

Associated features. The individual often displays signs of depression, anxiety, and guilt over the possibility or actuality of being apprehended and the resultant loss of status in society. Often, but not invariably, there are signs of personality disturbance.

Age at onset and course. The age at onset may be as early as childhood. The condition waxes and wanes and tends to be chronic; how often it "burns itself out" is unknown.

Impairment and complications. Impairment is usually due to the legal consequences of being apprehended, the major complication of the disorder.

Predisposing factors, prevalence, and familial pattern. No information.

Sex ratio. Although the majority of individuals apprehended for shop-

lifting are female, only a very small proportion of these individuals have Kleptomania. No data are available on the true sex ratio for the disorder.

Differential diagnosis. In **ordinary stealing** there is no evidence of a failure to resist the impulse; the act is usually planned, and the objects are stolen for their immediate use or monetary gain.

In **Malingering**, there may be an attempt to simulate the disorder in order to avoid criminal prosecution for common thievery. In **Conduct Disorder**, **Antisocial Personality Disorder**, and **manic episodes** stealing may occur; however, in such cases the act is obviously due to the more pervasive disorder.

In **Schizophrenia** stealing may be in response to delusions or hallucinations. In **Organic Mental Disorders** it may occur because of a failure to appreciate the consequences of the act, or because of failure to remember to pay for the object that has been taken.

Diagnostic criteria for Kleptomania

- A. Recurrent failure to resist impulses to steal objects that are not for immediate use or their monetary value.
- B. Increasing sense of tension before committing the act.
- C. An experience of either pleasure or release at the time of committing the theft.
- D. Stealing is done without long-term planning and assistance from, or collaboration with, others.
- E. Not due to Conduct Disorder or Antisocial Personality Disorder.

312.33 Pyromania

The essential features are recurrent failure to resist impulses to set fires and intense fascination with setting fires and seeing them burn. Before setting the fire, the individual experiences a buildup of tension; and once the fire is underway, he or she experiences intense pleasure or release. Although the fire-setting results from a failure to resist an impulse, there may be considerable advance preparation for starting the fire, and the individual may leave obvious clues.

The diagnosis is not made when fire-setting is due to Conduct Disorder, Antisocial Personality Disorder, Schizophrenia, or an Organic Mental Disorder.

Individuals with the disorder are often recognized as regular "watchers" at fires in their neighborhoods, frequently set off false alarms, and show interest in fire-fighting paraphernalia. They may be indifferent to the consequences of the fire for life or property, or they may get satisfaction from the resulting destruction.

Associated features. Alcohol Intoxication, Psychosexual Dysfunctions, lower than average IQ, chronic personal frustrations, and resentment of authority

figures are among the associated features. Cases have been described in which the individual was sexually aroused by fires.

Age at onset. Onset is usually in childhood. When it is in adolescence or adulthood, the fire-setting tends to be more deliberately destructive.

Course. No information.

Impairment and complications. Impairment is usually due to the legal consequences of being apprehended, the major complication of the disorder.

Sex ratio. The disorder is diagnosed far more commonly in males than in females.

Predisposing factors, prevalence, and familial pattern. No information.

Differential diagnosis. Young children's experimentation and fascination with matches, lighters, and fire may be a part of their normal investigation of their environment.

In **Conduct Disorder**, **Antisocial Personality Disorder**, and the **incendiary acts of sabotage** carried out by political extremists or by "paid torches," fire-setting occurs as a deliberate act rather than as a failure to resist an impulse. In **Schizophrenia**, fire-setting may be in response to delusions or hallucinations. In **Organic Mental Disorders**, fire-setting may occur because of failure to appreciate the consequences of the act.

Diagnostic criteria for Pyromania

- A. Recurrent failure to resist impulses to set fires.
- B. Increasing sense of tension before setting the fire.
- C. An experience of either intense pleasure, gratification, or release at the time of committing the act.
- D. Lack of motivation, such as monetary gain or sociopolitical ideology, for setting fires.
- E. Not due to an Organic Mental Disorder, Schizophrenia, Antisocial Personality Disorder, or Conduct Disorder.

312.34 Intermittent Explosive Disorder

The essential features are several discrete episodes of loss of control of aggressive impulses that result in serious assault or destruction of property. For example, with no or little provocation the individual may suddenly start to hit strangers and throw furniture. The degree of aggressivity expressed during an

episode is grossly out of proportion to any precipitating psychosocial stressor. The individual may describe the episodes as "spells" or "attacks." The symptoms appear within minutes or hours and, regardless of duration, remit almost as quickly. Genuine regret or self-reproach at the consequences of the action and the inability to control the aggressive impulse may follow each episode. There are no signs of generalized impulsivity or aggressiveness between the episodes.

The diagnosis is not made if the loss of control is due to Schizophrenia, Antisocial Personality Disorder, or Conduct Disorder. Mild forms of this disorder have, in the past, been called Explosive Personality.

Prodromal affective or autonomic symptoms may signal an impending episode. During the episode there may be subtle changes in sensorium; and following the episode there may be partial or spotty amnesia. The behavior is usually a surprise to those in the individual's milieu, and even the afflicted individual is often startled by his or her own behavior, sometimes describing the events as resulting from a compelling force beyond his or her control, even though he or she is willing to accept responsibility for his or her actions.

Associated features. It is not clear to what extent other associated psychopathology is usually present between episodes.

Often individuals claim hypersensitivity to sensory input such as loud noises, rhythmic auditory or visual stimuli, and bright lights. Other features suggesting an organic disturbance may be present, such as nonspecific EEG abnormalities or minor neurological signs and symptoms thought to reflect subcortical or limbic system dysfunction. Epilepsy is rarely present, but is nevertheless more common than in individuals without the disorder. Medical history often reveals hyperactive motor behavior and proneness to accident.

Age at onset. The disorder may begin at any stage of life, but more commonly begins in the second or third decade.

Course. No information.

Impairment. Normal social relations may be impaired because of social ostracism that results from the unpredictable aggressive behavior.

Complications. Incarceration or chronic hospitalization may result.

Predisposing factors. Any toxic agent, such as alcohol, that may lower the threshold for violent outbursts, and conditions conducive to brain dysfunction, such as perinatal trauma, infantile seizures, head trauma, and encephalitis may predispose to this disorder.

Prevalence. The disorder is apparently very rare.

Sex ratio. The disorder is apparently more common in males than in females. The males are likely to be seen in a correctional institution and the females, in a mental health facility.

Familial pattern. The disorder is apparently more common in family members than in the general population.

Differential diagnosis. An **underlying physical disorder**, such as a brain tumor or epilepsy, may in rare cases cause this syndrome. In such instances the diagnosis Intermittent Explosive Disorder should be recorded on Axis I, and the physical disorder, on Axis III.

In **Antisocial Personality Disorder**, outbursts of aggressiveness are common, but aggressiveness and impulsivity are also present between the outbursts. In **Dissociative Disorder** any loss of control that occurs invariably follows a major stressful event, whereas in this disorder there is usually only a minor or no precipitating event. In any case, if the disturbance meets the criteria for Intermittent Explosive Disorder, this precludes a diagnosis of a Dissociative Disorder.

In **Paranoid Disorder** or **Schizophrenia, Catatonic Type**, there may be outbursts of violent behavior in response to delusions or hallucinations.

Diagnostic criteria for Intermittent Explosive Disorder

A. Several discrete episodes of loss of control of aggressive impulses resulting in serious assault or destruction of property.

B. Behavior that is grossly out of proportion to any precipitating psychosocial stressor.

C. Absence of signs of generalized impulsivity or aggressiveness between episodes.

D. Not due to Schizophrenia, Antisocial Personality Disorder, or Conduct Disorder.

312.35 Isolated Explosive Disorder

The essential feature is a single, discrete episode of failure to resist an impulse that led to a single, violent, externally directed act, which had a catastrophic impact on others and for which the available information does not justify the diagnosis of Schizophrenia, Antisocial Personality Disorder, or Conduct Disorder. An example would be an individual who for no apparent reason suddenly began shooting at total strangers in a fit of rage and then shot himself. In the past this disorder was referred to as "catathymic crisis."

In some cases additional information indicates an underlying psychosis, such as Schizophrenia, Paranoid Type, which would then preempt this diagnosis. As with Intermittent Explosive Disorder, this category is defined behaviorally. In those rare instances in which an underlying organic etiology is revealed, such as a brain tumor, this would be an additional diagnosis, coded on Axis III.

Other features of this disorder are similar to those of Intermittent Explosive Disorder.

Diagnostic criteria for Isolated Explosive Disorder

- A. A single, discrete episode in which failure to resist an impulse led to a single, violent, externally directed act that had a catastrophic impact on others.
- B. The degree of aggressivity expressed during the episode was grossly out of proportion to any precipitating psychosocial stressor.
- C. Before the episode there were no signs of generalized impulsivity or aggressiveness.
- D. Not due to Schizophrenia, Antisocial Personality Disorder, or Conduct Disorder.

312.39 Atypical Impulse Control Disorder

This category is for Disorders of Impulse Control that cannot be classified elsewhere.

Adjustment Disorder

The essential feature is a maladaptive reaction to an identifiable psychosocial stressor, that occurs within three months after the onset of the stressor. The maladaptive nature of the reaction is indicated by either impairment in social or occupational functioning or symptoms that are in excess of a normal and expected reaction to the stressor. The disturbance is not merely one instance of a pattern of overreaction to a stressor or an exacerbation of one of the mental disorders previously described. It is assumed that the disturbance will eventually remit after the stressor ceases or, if the stressor persists, when a new level of adaptation is achieved. This category should not be used if the disturbance meets the criteria for a specific disorder, such as an Anxiety or Affective Disorder.

The stressors may be single, such as divorce, or multiple, such as marked business difficulties and marital problems. They may be recurrent, as with seasonal business crises, or continuous, as with chronic illness or residence in a deteriorating neighborhood. They can occur in a family setting, e.g., in discordant intrafamilial relationships. They may affect only the individual, e.g., the psychological reaction to a physical illness, or they may affect a group or community, e.g., a natural disaster, or persecution based on racial, social, religious, or other group affiliation. Some stressors are associated with specific developmental stages, such as going to school, leaving the parental home, getting married, becoming a parent, failing to attain occupational goals, and retirement.

The severity of the stressor and the specific stressor may be noted on Axis IV (p. 26). The severity of a specific stressor is affected by its duration, timing, and context in a person's life. For example, the stress of losing a parent is different for a child and an adult.

The severity of the reaction is not completely predictable from the severity of the stressor. Individuals who are particularly vulnerable may have a more severe form of the disorder following only a mild or moderate stressor, whereas others may have only a mild form of the disorder in response to a marked and continuing stressor.

Types. The manifestations of the disorder are varied. Each specific type represents a predominant clinical picture (p. 301), many of which are partial syndromes of specific disorders. For example, Adjustment Disorder with Depressed Mood is manifested by an incomplete depressive syndrome in response to a psychosocial stressor.

Age at onset. Adjustment Disorder may begin at any age.

Course. By definition the disturbance begins within three months of the

onset of the stressor. If the stressor is a discrete event, such as being fired from a job, the onset of the disturbance is usually within a few days, and the duration is relatively brief—no more than a few months. If the stressor continues, as with a chronic physical illness, the duration may be much longer until a new level of adaptation is achieved.

Predisposing factors. A preexisting Personality Disorder or Organic Mental Disorder may increase an individual's vulnerability to stress and predispose to the development of Adjustment Disorder.

Prevalence. The disorder is apparently common.

Sex ratio and familial pattern. No information.

Differential diagnosis. In **Conditions Not Attributable to a Mental Disorder** (V codes), such as Other Interpersonal Problem or Phase of Life Problem or Other Life Circumstance Problem, there is neither impairment in social or occupational functioning nor symptoms that are in excess of a normal and expectable reaction to the stressor. No absolute guidelines are available to aid in this fundamental distinction, so clinical judgment will often be required.

Personality Disorders are often repeatedly exacerbated by stress, in which case the additional diagnosis of Adjustment Disorder is not made. However, if new features are seen in response to a stressor—such as depressed mood in an individual with Paranoid Personality Disorder who has never been bothered by depression—then the additional diagnosis of Adjustment Disorder may be appropriate.

In **Psychological Factors Affecting Physical Condition** the individual may be reacting to a psychosocial stressor, but the predominant symptomatology is a physical condition or disorder.

Diagnostic criteria for Adjustment Disorder

A. A maladaptive reaction to an identifiable psychosocial stressor, that occurs within three months of the onset of the stressor.

B. The maladaptive nature of the reaction is indicated by either of the following:

- (1) impairment in social or occupational functioning
- (2) symptoms that are in excess of a normal and expectable reaction to the stressor

C. The disturbance is not merely one instance of a pattern of overreaction to stress or an exacerbation of one of the mental disorders previously described.

D. It is assumed that the disturbance will eventually remit after the stressor ceases or, if the stressor persists, when a new level of adaptation is achieved.

E. The disturbance does not meet the criteria for any of the specific disorders listed previously or for Uncomplicated Bereavement.

TYPES OF ADJUSTMENT DISORDER. Code predominant symptoms.

309.00 Adjustment Disorder with Depressed Mood

This category should be used when the predominant manifestation involves such symptoms as depressed mood, tearfulness, and hopelessness. The major differential is with Major Depression and Uncomplicated Bereavement.

309.24 Adjustment Disorder with Anxious Mood

This category should be used when the predominant manifestation involves such symptoms as nervousness, worry, and jitteriness. The major differential is with Anxiety Disorders.

309.28 Adjustment Disorder with Mixed Emotional Features

This category should be used when the predominant manifestation involves various combinations of depression and anxiety or other emotions. The major differential is with Depressive and Anxiety Disorders. An example would be an adolescent, after moving away from home and parental supervision, who reacts with ambivalence, depression, anger, and signs of increased dependency.

309.30 Adjustment Disorder with Disturbance of Conduct

This category should be used when the predominant manifestation involves conduct in which there is violation of the rights of others or of major age-appropriate societal norms and rules. Examples: truancy, vandalism, reckless driving, fighting, defaulting on legal responsibilities. The major differential is with Conduct Disorder and Antisocial Personality Disorder.

309.40 Adjustment Disorder with Mixed Disturbance of Emotions and Conduct

This category should be used when the predominant manifestation involves both emotional features (e.g., depression, anxiety) and a disturbance of conduct (see above).

309.23 Adjustment Disorder with Work (or Academic) Inhibition

This category should be used when the predominant manifestation is an inhibition in work or academic functioning occurring in an individual whose previous work or academic performance has been adequate. Frequently there are also varying mixtures of anxiety and depression. Examples include inability to study and to write papers or reports. The major differential is with Depressive Disorders and Anxiety Disorders.

309.83 Adjustment Disorder with Withdrawal

This category should be used when the predominant manifestation involves social withdrawal without significant depressed or anxious mood. The major differential is with Depressive Disorders.

309.90 Adjustment Disorder with Atypical Features

This category should be used when the predominant manifestation involves symptoms that cannot be coded in any of the specific categories.

Psychological Factors Affecting Physical Condition

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This category enables a clinician to note that psychological factors contribute to the initiation or exacerbation of a physical condition. The physical condition will usually be a physical disorder, but in some instances may be only a single symptom, such as vomiting. The physical condition should be recorded on Axis III.

This manual accepts the tradition of referring to certain factors as "psychological," although it is by no means easy to define what this phrase means. A limited but useful definition in this context is the meaning ascribed to environmental stimuli by the individual. Common examples of such stimuli are the sights and sounds arising in interpersonal transactions, such as arguments, and information that a loved one has died. The individual may not be aware of the meaning that he or she has given to such environmental stimuli or of the relationship between these stimuli and the initiation or exacerbation of the physical condition.

The judgment that psychological factors are affecting the physical condition requires evidence of a temporal relationship between the environmental stimuli and the meaning ascribed to them and the initiation or exacerbation of the physical condition. Obviously, this judgment is more certain when there are repeated instances of a temporal relationship.

This category can be used for any physical condition to which psychological factors are judged to be contributory. It can be used to describe disorders that in the past have been referred to as either "psychosomatic" or "psychophysiological."

Common examples of physical conditions for which this category may be appropriate include, but are not limited to: obesity, tension headache, migraine headache, angina pectoris, painful menstruation, sacroiliac pain, neurodermatitis, acne, rheumatoid arthritis, asthma, tachycardia, arrhythmia, gastric ulcer, duodenal ulcer, cardiospasm, pylorospasm, nausea and vomiting, regional enteritis, ulcerative colitis, and frequency of micturition.

This category should not be used for Conversion Disorders, which are regarded as disturbances in which the specific pathophysiological process involved in the disorder is not demonstrable by existing standard laboratory procedures and which are conceptualized with psychological constructs only.

Diagnostic criteria for Psychological Factors Affecting Physical Condition

A. Psychologically meaningful environmental stimuli are temporally

related to the initiation or exacerbation of a physical condition (recorded on Axis III).

B. The physical condition has either demonstrable organic pathology (e.g., rheumatoid arthritis) or a known pathophysiological process (e.g., migraine headache, vomiting).

C. The condition is not due to a Somatoform Disorder.

Personality Disorders

(NOTE: These are coded on Axis II.)

Personality *traits* are enduring patterns of perceiving, relating to, and thinking about the environment and oneself, and are exhibited in a wide range of important social and personal contexts. It is only when *personality traits* are inflexible and maladaptive and cause either significant impairment in social or occupational functioning or subjective distress that they constitute *Personality Disorders*. The manifestations of Personality Disorders are generally recognizable by adolescence or earlier and continue throughout most of adult life, though they often become less obvious in middle or old age.

Many of the features characteristic of the various Personality Disorders, such as Dependent, Paranoid, Schizotypal, or Borderline Personality Disorder, may be seen during an episode of another mental disorder, such as Major Depression. The diagnosis of a Personality Disorder should be made only when the characteristic features are typical of the individual's long-term functioning and are not limited to discrete episodes of illness.

DIAGNOSIS OF PERSONALITY DISORDERS IN CHILDREN AND ADOLESCENTS

Certain Personality Disorders have a relationship to corresponding diagnostic categories in the section Disorders Usually First Evident in Infancy, Childhood, or Adolescence. The corresponding disorders are as follows:

Disorders of Childhood or Adolescence

Schizoid Disorder of Childhood or Adolescence

Avoidant Disorder of Childhood or Adolescence

Conduct Disorder

Oppositional Disorder

Identity Disorder

Personality Disorders

Schizoid Personality Disorder

Avoidant Personality Disorder

Antisocial Personality Disorder

Passive-Aggressive Personality Disorder

Borderline Personality Disorder

If the features of Schizoid or Avoidant Disorder of Childhood or Adolescence continue into adult life, i.e., after age 18, the diagnosis should be changed to the corresponding Personality Disorder. Some children or adolescents with Conduct Disorder, particularly the Undersocialized Aggressive type, may meet the criteria for Antisocial Personality Disorder in adulthood, in which case the diagnosis should be changed. Similarly, some children with Oppositional Dis-

order as adults may have a disorder meeting the criteria for Passive-Aggressive Personality Disorder, and some cases of Identity Disorder may later meet the criteria for Borderline Personality Disorder.

The other Personality Disorder categories may be applied to children or adolescents in those unusual instances in which the particular maladaptive personality traits appear to be stable. When this is done, there is obviously less certainty that the Personality Disorder will persist unchanged over time.

Associated features. Frequently the individual with a Personality Disorder is dissatisfied with the impact his or her behavior is having on others or with his or her inability to function effectively. This may be the case even when the traits that lead to these difficulties are ego-syntonic, that is, are not regarded by the individual as undesirable. In other cases, the traits may be ego-dystonic, but the individual may be unable to modify them despite great effort.

Disturbances of mood, frequently involving depression or anxiety, are common, and may even be the individual's chief complaint.

Age at onset and course. As noted above, Personality Disorders by definition begin in childhood or adolescence and are characteristic of most of adult life.

Impairment. Marked impairment in social and occupational functioning may exist. When occupational functioning is impaired, the impairment is usually sustained, but may be episodic and take the form of recurrent periods of work inhibition (e.g., "writer's block"). With the exception of Antisocial, Schizotypal, and Borderline Personality Disorders, individuals with Personality Disorders rarely require hospitalization unless there is a superimposed disorder, such as a Substance Use Disorder or Major Depression.

PERSONALITY DISORDERS AND PSYCHOTIC DISORDERS

When an individual with a psychotic disorder coded on Axis I, for example, Schizophrenia or Paranoia, has had a preexisting Personality Disorder, the Personality Disorder should also be recorded on Axis II, followed by "Premorbid" in parentheses. For example:

Axis I: 295.32 Schizophrenia, Paranoid, Chronic

Axis II: 301.20 Schizoid Personality Disorder (Premorbid)

SPECIFIC PERSONALITY DISORDERS

Traditionally, in diagnosing Personality Disorders the clinician has been directed to find a single, specific Personality Disorder that adequately describes the individual's disturbed personality functioning. Frequently this can be done only with difficulty, since many individuals exhibit features that are not limited to a single Personality Disorder. In this manual diagnoses of more than one Personality Disorder should be made if the individual meets the criteria for more than one.

The Personality Disorders have been grouped into three clusters. The first cluster includes Paranoid, Schizoid, and Schizotypal Personality Disorders. Individuals with these disorders often appear "odd" or eccentric. The second cluster includes Histrionic, Narcissistic, Antisocial, and Borderline Personality Disorders. Individuals with these disorders often appear dramatic, emotional, or erratic. The third cluster includes Avoidant, Dependent, Compulsive, and Passive-Aggressive Personality Disorders. Individuals with these disorders often appear anxious or fearful. Finally, there is a residual category, Atypical, Mixed, or Other Personality Disorder, that can be used for other specific Personality Disorders or for conditions that do not qualify as any of the specific Personality Disorders described in this manual.

There is great variability in the detail with which the various Personality Disorders are described and the specificity of the diagnostic criteria. Disorders studied more extensively and rigorously than others, such as Antisocial Personality Disorder, are described in greater detail.

301.00 Paranoid Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there is a pervasive and unwarranted suspiciousness and mistrust of people, hypersensitivity, and restricted affectivity not due to another mental disorder, such as Schizophrenia, or a Paranoid Disorder.

An attitude of suspicion is not only justified but is adaptive in many difficult life situations. A person without signs of mental disorder is willing in such a situation to abandon suspicions when presented with convincing contradictory evidence, but one with Paranoid Personality Disorder ignores such evidence, and may even become suspicious of someone who challenges his or her suspicious ideas. Individuals with this disorder are typically hypervigilant and take precautions against any perceived threat. They tend to avoid blame even when it is warranted. They are often viewed by others as guarded, secretive, devious, and scheming. They may question the loyalty of others, always expecting trickery. For this reason, there may be pathological jealousy.

When individuals with this disorder find themselves in a new situation, they intensely and narrowly search for confirmation of their expectations, with no appreciation of the total context. Their final conclusion is usually precisely what they expected in the first place. They are concerned with hidden motives and special meanings. Often, transient ideas of reference occur, e.g., that others are taking special notice of them, or saying vulgar things about them.

Individuals with this disorder are usually argumentative and exaggerate difficulties by "making mountains out of molehills." They often find it difficult to relax, usually appear tense, and show a tendency to counterattack when they perceive any threat. Though they are critical of others, and often litigious, they have great difficulty accepting criticism themselves.

These individuals' affectivity is restricted, and they may appear "cold" to others. They have no true sense of humor and are usually serious. They may pride themselves on always being objective, rational, and unemotional. They usually lack passive, soft, sentimental, and tender feelings.

Associated features. Individuals with this disorder are occasionally seen by others as keen observers who are energetic, ambitious, and capable; but more often they are viewed as hostile, stubborn, and defensive. They tend to be rigid and unwilling to compromise. They often generate uneasiness and fear in others. Often there is an inordinate fear of losing independence or the power to shape events in accordance with their own wishes.

They usually avoid intimacy except with those in whom they have absolute trust. They show an excessive need to be self-sufficient, to the point of egocentricity and exaggerated self-importance. They avoid participation in group activities unless they are in a dominant position.

They are often interested in mechanical devices, electronics, and automation. They are keenly aware of power and rank and of who is superior or inferior, and are often envious and jealous of those in positions of power. They disdain people seen as weak, soft, sickly, or defective. They are generally uninterested in art or aesthetics.

During periods of extreme stress, transient psychotic symptoms may occur, but of insufficient severity or duration to warrant an additional diagnosis.

Impairment. Because individuals with this disorder usually realize that it is prudent to keep their unusual ideas to themselves, impairment is generally minimal. However, occupational difficulties are common, especially in relating to authority figures or co-workers. In more severe cases, all relationships are grossly impaired.

Complications. The relationship of this disorder to the Paranoid Disorders (p. 196) and Schizophrenia, Paranoid Type (p. 188), is unclear. However, certain of the essential features of Paranoid Personality Disorder, such as suspiciousness and hypersensitivity, may predispose to the development of these other disorders.

Predisposing factors. No information.

Prevalence. This disorder rarely comes to clinical attention, since such persons rarely seek help for their personality problems or require hospitalization. Owing to a tendency of some of them to be moralistic, grandiose, and extrapunitive, it seems likely that individuals with this disorder are overrepresented among leaders of mystical or esoteric religions and of pseudoscientific and quasi-political groups.

Sex ratio. This disorder is more commonly diagnosed in men.

Familial pattern. No information.

Differential diagnosis. In **Paranoid Disorders** and **Schizophrenia, Paranoid Type**, there are persistent psychotic symptoms, such as delusions and hallucinations, that are never part of Paranoid Personality Disorder. However, these disorders may be superimposed on Paranoid Personality Disorder. **Antisocial Personality Disorder** shares several features with Paranoid Personality Disorder,

e.g., difficulty in forming and sustaining close relationships, and poor occupational performance; but except when the two disorders coexist, a lifelong history of antisocial behavior is not present in Paranoid Personality Disorder.

Diagnostic criteria for Paranoid Personality Disorder

The following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.

A. Pervasive, unwarranted suspiciousness and mistrust of people as indicated by at least three of the following:

- (1) expectation of trickery or harm
- (2) hypervigilance, manifested by continual scanning of the environment for signs of threat, or taking unneeded precautions
- (3) guardedness or secretiveness
- (4) avoidance of accepting blame when warranted
- (5) questioning the loyalty of others
- (6) intense, narrowly focused searching for confirmation of bias, with loss of appreciation of total context
- (7) overconcern with hidden motives and special meanings
- (8) pathological jealousy

B. Hypersensitivity as indicated by at least two of the following:

- (1) tendency to be easily slighted and quick to take offense
- (2) exaggeration of difficulties, e.g., "making mountains out of mole-hills"
- (3) readiness to counterattack when any threat is perceived
- (4) inability to relax

C. Restricted affectivity as indicated by at least two of the following:

- (1) appearance of being "cold" and unemotional
- (2) pride taken in always being objective, rational, and unemotional
- (3) lack of a true sense of humor
- (4) absence of passive, soft, tender, and sentimental feelings

D. Not due to another mental disorder such as Schizophrenia or a Paranoid Disorder.

Schizoid Personality Disorder

Schizotypal Personality Disorder

Schizoid and Schizotypal Personality Disorders are new diagnostic categories.

In the past, the term Schizoid was applied to individuals with defects in the capacity to form social relationships. In addition, the term was applied to individuals with various eccentricities of communication or behavior. Because recent evidence suggests a possible relationship between the latter group of individuals and a family history of chronic Schizophrenia, they are diagnosed in this manual separately as having Schizotypal Personality Disorder (see p. 312). The term Schizotypal is given to this category because, in addition, the features of this disorder are frequently present in individuals with Schizophrenia, Residual Type.

Some cases previously diagnosed as Borderline, Latent, or Simple Schizophrenia are likely to be classified in this manual as Schizotypal Personality Disorder.

Individuals with defects in the capacity to form social relationships but without eccentricities of communication or behavior are diagnosed here as having Schizoid Personality Disorder, even though the term is more inclusive in other classifications and suggests a relationship to Schizophrenia.

301.20 Schizoid Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there is a defect in the capacity to form social relationships, evidenced by the absence of warm, tender feelings for others and indifference to praise, criticism, and the feelings of others. The diagnosis is not made if eccentricities of speech, behavior, or thought characteristic of Schizotypal Personality Disorder are present or if the disturbance is due to a psychotic disorder such as Schizophrenia.

Individuals with this disorder show little or no desire for social involvement, usually prefer to be "loners," and have few, if any, close friends. They appear reserved, withdrawn, and seclusive and usually pursue solitary interests or hobbies. Individuals with this disorder are usually humorless or dull and without affect in situations in which an emotional response would be appropriate. They usually appear "cold" and aloof.

Associated features. Individuals with this disorder are often unable to express aggressiveness or hostility. They may seem vague about their goals, indecisive in their actions, self-absorbed, absentminded, and detached from their environment ("not with it" or "in a fog"). Excessive daydreaming is often present.

Because of a lack of social skills, males with this disorder are usually incapable of dating and rarely marry. Females may passively accept courtship and marry.

Impairment. Social relations are, by definition, severely restricted. Occupational functioning may be impaired, particularly if interpersonal involvement is required. On the other hand, individuals with this disorder may, in some instances, be capable of high occupational achievement in situations requiring work performance under conditions of social isolation.

Complications. Some believe that Schizophrenia may develop as a complication of this disorder, although others believe that in such cases, the Schizoid

Personality Disorder merely represented the prodromal phase of the Schizophrenia.

Predisposing factors. Schizoid Disorder of Childhood or Adolescence predisposes to the development of Schizoid Personality Disorder.

Prevalence. Although the prevalence is unknown, a significant proportion of individuals working in jobs that involve little or no contact with others, or living in skid-row sections of cities may have this disorder.

Sex ratio and familial pattern. No information.

Differential diagnosis. In **Schizotypal Personality Disorder** there are eccentricities of communication or behavior, and these preclude a diagnosis of Schizoid Personality Disorder.

In **Avoidant Personality Disorder**, social isolation is due to hypersensitivity to rejection, but a desire to enter social relationships is present if there are strong guarantees of uncritical acceptance. In contrast, individuals with Schizoid Personality Disorder have no desire for social relations.

In **Schizoid Disorder of Childhood or Adolescence** there is a similar clinical picture and this diagnosis preempts the diagnosis of Schizoid Personality Disorder if the individual is under 18.

Diagnostic criteria for Schizoid Personality Disorder

The following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.

A. Emotional coldness and aloofness, and absence of warm, tender feelings for others.

B. Indifference to praise or criticism or to the feelings of others.

C. Close friendships with no more than one or two persons, including family members.

D. No eccentricities of speech, behavior, or thought characteristic of Schizotypal Personality Disorder.

E. Not due to a psychotic disorder such as Schizophrenia or Paranoid Disorder.

F. If under 18, does not meet the criteria for Schizoid Disorder of Childhood or Adolescence.

301.22 Schizotypal Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there are various oddities of thought, perception, speech, and behavior that are not severe enough to meet the criteria for Schizophrenia. No single feature is invariably present. The disturbance in the content of thought may include magical thinking (or in children, bizarre fantasies or preoccupations), ideas of reference, or paranoid ideation. Perceptual disturbances may include recurrent illusions, depersonalization, or derealization (not associated with panic attacks). Often, speech shows marked peculiarities: concepts may be expressed unclearly or oddly or words used deviantly, but never to the point of loosening of associations or incoherence. Frequently, but not invariably, the behavioral manifestations include social isolation and constricted or inappropriate affect that interferes with rapport in face-to-face interaction.

Associated features. Varying admixtures of anxiety, depression, and other dysphoric moods are common. Features of Borderline Personality Disorder (p. 321) are often present, and in some cases both diagnoses may be warranted. During periods of extreme stress transient psychotic symptoms may be present. Because of peculiarities in thinking, individuals with Schizotypal Personality Disorder are prone to eccentric convictions, such as bigotry and fringe religious beliefs.

Impairment. Usually some interference with social or occupational functioning occurs.

Complications. Psychotic disorders such as Brief Reactive Psychosis may occur.

Prevalance and sex ratio. No information.

Familial pattern. There is some evidence that chronic Schizophrenia is more common among family members of individuals with Schizotypal Personality Disorder than among the general population.

Differential diagnosis. In **Schizophrenia, Residual Type**, there is a history of an active phase of Schizophrenia with psychotic symptoms. When psychotic symptoms occur in Schizotypal Personality Disorder, they are transient and not as severe. In **Schizoid Personality Disorder** and **Avoidant Personality Disorder**, there are no oddities of behavior, thinking, perception, and speech as are present in Schizotypal Personality Disorder. Frequently, individuals with **Borderline Personality Disorder** also meet the criteria for Schizotypal Personality Disorder; in such instances, both diagnoses should be recorded. In **Depersonalization Disorder**, oddities of thought, speech, and behavior are not present, although in rare cases both disorders may coexist.

Diagnostic criteria for Schizotypal Personality Disorder

The following are characteristic of the individual's current and long-term

functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.

A. At least four of the following:

- (1) magical thinking, e.g., superstitiousness, clairvoyance, telepathy, "6th sense," "others can feel my feelings" (in children and adolescents, bizarre fantasies or preoccupations)
- (2) ideas of reference
- (3) social isolation, e.g., no close friends or confidants, social contacts limited to essential everyday tasks
- (4) recurrent illusions, sensing the presence of a force or person not actually present (e.g., "I felt as if my dead mother were in the room with me"), depersonalization, or derealization not associated with panic attacks
- (5) odd speech (without loosening of associations or incoherence), e.g., speech that is digressive, vague, overelaborate, circumstantial, metaphorical
- (6) inadequate rapport in face-to-face interaction due to constricted or inappropriate affect, e.g., aloof, cold
- (7) suspiciousness or paranoid ideation
- (8) undue social anxiety or hypersensitivity to real or imagined criticism

B. Does not meet the criteria for Schizophrenia.

301.50 Histrionic Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there are overly dramatic, reactive, and intensely expressed behavior and characteristic disturbances in interpersonal relationships.

Individuals with this disorder are lively and dramatic and are always drawing attention to themselves. They are prone to exaggeration and often act out a role, such as the "victim" or the "princess," without being aware of it.

Behavior is overly reactive and intensely expressed. Minor stimuli give rise to emotional excitability, such as irrational, angry outbursts or tantrums. Individuals with this disorder crave novelty, stimulation, and excitement and quickly become bored with normal routines.

Interpersonal relationships show characteristic disturbances. Initially, people with this disorder are frequently perceived as shallow and lacking genuineness, though superficially charming and appealing. They are often quick to form friendships; but once a relationship is established they can become demanding, egocentric, and inconsiderate; manipulative suicidal threats, gestures, or attempts may be made; there may be a constant demand for reassurance because of feelings of helplessness and dependency. In some cases both patterns are

present in the same relationship. These people's actions are frequently inconsistent, and may be misinterpreted by others.

Such individuals are typically attractive and seductive. They attempt to control the opposite sex or enter into a dependent relationship. Flights into romantic fantasy are common; in both sexes overt behavior often is a caricature of femininity. The actual quality of their sexual relationships is variable. Some individuals are promiscuous; others, naïve and sexually unresponsive; but still others have apparently normal sexual adjustment.

In other classifications this category is termed *Hysterical Personality*.

Associated features. Individuals with this disorder often experience periods of intense dissatisfaction and a variety of dysphoric moods, usually related to obvious changes in external circumstances, such as a breakup with a lover. They may make suicidal gestures or attempts.

Usually these individuals show little interest in intellectual achievement and careful, analytic thinking, though they are often creative and imaginative.

Individuals with this disorder tend to be impressionable and easily influenced by others or by fads. They are apt to be overly trusting of others, suggestible, and show an initially positive response to any strong authority figure who they think can provide a magical solution for their problems. Though they adopt convictions strongly and readily, their judgment is not firmly rooted, and they often play hunches.

Frequent complaints of poor health, such as weakness or headaches, or subjective feelings of depersonalization may be present. During periods of extreme stress, there may be transient psychotic symptoms of insufficient severity or duration to warrant an additional diagnosis.

When the disorder is present in men, it is sometimes associated with a homosexual arousal pattern.

Impairment. Interpersonal relations are usually stormy and ungratifying. In extreme cases there is gross inability to function.

Complications. A common complication is Substance Use Disorder, particularly in women. Additional complications include Major Depression, Dysthymic Disorder, Brief Reactive Psychosis, Conversion Disorder, and Somatization Disorder.

Predisposing factors. No information.

Prevalence and sex ratio. The disorder is apparently common, and diagnosed far more frequently in females than in males.

Familial pattern. The disorder is apparently more common among family members than in the general population.

Differential diagnosis. In **Somatization Disorder** complaints of physical illness dominate the clinical picture, although histrionic features are common. In many cases Somatization Disorder and Histrionic Personality Disorder coexist.

Borderline Personality Disorder is also often present; in such cases both diagnoses should be made.

Diagnostic criteria for Histrionic Personality Disorder

The following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.

A. Behavior that is overly dramatic, reactive, and intensely expressed, as indicated by at least three of the following:

- (1) self-dramatization, e.g., exaggerated expression of emotions
- (2) incessant drawing of attention to oneself
- (3) craving for activity and excitement
- (4) overreaction to minor events
- (5) irrational, angry outbursts or tantrums

B. Characteristic disturbances in interpersonal relationships as indicated by at least two of the following:

- (1) perceived by others as shallow and lacking genuineness, even if superficially warm and charming
- (2) egocentric, self-indulgent, and inconsiderate of others
- (3) vain and demanding
- (4) dependent, helpless, constantly seeking reassurance
- (5) prone to manipulative suicidal threats, gestures, or attempts

301.81 Narcissistic Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there are a grandiose sense of self-importance or uniqueness; preoccupation with fantasies of unlimited success; exhibitionistic need for constant attention and admiration; characteristic responses to threats to self-esteem; and characteristic disturbances in interpersonal relationships, such as feelings of entitlement, interpersonal exploitativeness, relationships that alternate between the extremes of overidealization and devaluation, and lack of empathy.

The exaggerated sense of self-importance may be manifested as extreme self-centeredness and self-absorption. Abilities and achievements tend to be unrealistically overestimated. Frequently the sense of self-importance alternates with feelings of special unworthiness. For example, a student who ordinarily expects an A and receives an A minus may at that moment express the view that he or she, more than any other student, is revealed to all as a failure.

Fantasies involving unrealistic goals may involve achieving unlimited ability, power, wealth, brilliance, beauty, or ideal love. Although these fantasies

frequently substitute for realistic activity, when these goals are actually pursued, it is often with a "driven," pleasureless quality, and an ambition that cannot be satisfied.

Individuals with this disorder are constantly seeking admiration and attention, and are more concerned with appearances than with substance. For example, there might be more concern about being seen with the "right" people than having close friends.

Self-esteem is often fragile; the individual may be preoccupied with how well he or she is doing and how well he or she is regarded by others. In response to criticism, defeat, or disappointment, there is either a cool indifference or marked feelings of rage, inferiority, shame, humiliation, or emptiness.

Interpersonal relationships are invariably disturbed. A lack of empathy (inability to recognize and experience how others feel) is common. For example, annoyance and surprise may be expressed when a friend who is seriously ill has to cancel a date.

Entitlement, the expectation of special favors without assuming reciprocal responsibilities, is usually present. For example, surprise and anger are felt because others will not do what is wanted; more is expected from people than is reasonable.

Interpersonal exploitativeness, in which others are taken advantage of in order to indulge one's own desires or for self-aggrandizement, is common; and the personal integrity and rights of others are disregarded. For example, a writer might plagiarize the ideas of someone befriended for that purpose.

Relations with others lack sustained, positive regard. Close relationships tend to alternate between idealization and devaluation ("splitting"). For example, a man repeatedly becomes involved with women whom he alternately adores and despises.

Associated features. Frequently, many of the features of Histrionic, Borderline, and Antisocial Personality Disorders are present; in some cases more than one diagnosis may be warranted.

During periods of severe stress transient psychotic symptoms of insufficient severity or duration to warrant an additional diagnosis are sometimes seen.

Depressed mood is extremely common. Frequently there is painful self-consciousness, preoccupation with grooming and remaining youthful, and chronic, intense envy of others. Preoccupation with aches and pains and other physical symptoms may also be present. Personal deficits, defeats, or irresponsible behavior may be justified by rationalization, prevarication, or outright lying. Feelings may be faked in order to impress others.

Impairment. By definition, some impairment in interpersonal relations always exists. Occupational functioning may be unimpaired, or may be interfered with by depressed mood, interpersonal difficulties, or the pursuit of unrealistic goals.

Complications. Dysthymic Disorder, Major Depression and psychotic disorders such as Brief Reactive Psychosis are possible complications.

Prevalence. This disorder appears to be more common recently than in the past, although this may only be due to greater professional interest in the category.

Predisposing factors, sex ratio, and familial pattern. No information.

Differential diagnosis. **Borderline and Histrionic Personality Disorders** are often also present; in such instances, multiple diagnoses should be given.

Diagnostic criteria for Narcissistic Personality Disorder

The following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress:

- A. Grandiose sense of self-importance or uniqueness, e.g., exaggeration of achievements and talents, focus on the special nature of one's problems.
- B. Preoccupation with fantasies of unlimited success, power, brilliance, beauty, or ideal love.
- C. Exhibitionism: the person requires constant attention and admiration.
- D. Cool indifference or marked feelings of rage, inferiority, shame, humiliation, or emptiness in response to criticism, indifference of others, or defeat.
- E. At least two of the following characteristic of disturbances in interpersonal relationships:
 - (1) entitlement: expectation of special favors without assuming reciprocal responsibilities, e.g., surprise and anger that people will not do what is wanted
 - (2) interpersonal exploitativeness: taking advantage of others to indulge own desires or for self-aggrandizement; disregard for the personal integrity and rights of others
 - (3) relationships that characteristically alternate between the extremes of overidealization and devaluation
 - (4) lack of empathy: inability to recognize how others feel, e.g., unable to appreciate the distress of someone who is seriously ill.

301.70 Antisocial Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there are a history of continuous and chronic antisocial behavior in which the rights of

others are violated, persistence into adult life of a pattern of antisocial behavior that began before the age of 15, and failure to sustain good job performance over a period of several years (although this may not be evident in individuals who are self-employed or who have not been in a position to demonstrate this feature, e.g., students or housewives). The antisocial behavior is not due to either severe Mental Retardation, Schizophrenia, or manic episodes.

Lying, stealing, fighting, truancy, and resisting authority are typical early childhood signs. In adolescence, unusually early or aggressive sexual behavior, excessive drinking, and use of illicit drugs are frequent. In adulthood, these kinds of behavior continue, with the addition of inability to sustain consistent work performance or to function as a responsible parent and failure to accept social norms with respect to lawful behavior. After age 30 the more flagrant aspects may diminish, particularly sexual promiscuity, fighting, criminality, and vagrancy.

Associated features. Despite the stereotype of a normal mental status in this disorder, frequently there are signs of personal distress, including complaints of tension, inability to tolerate boredom, depression, and the conviction (often correct) that others are hostile toward them. The interpersonal difficulties and dysphoria tend to persist into late adult life even when the more flagrant antisocial behavior has diminished. Almost invariably there is markedly impaired capacity to sustain lasting, close, warm, and responsible relationships with family, friends, or sexual partners.

Impairment. The disorder is often extremely incapacitating, resulting in failure to become an independent, self-supporting adult and in many years of institutionalization, more commonly penal than medical. It is possible, however, for individuals who have some of the features of the disorder to achieve political and economic success; but these people virtually never present the full picture of the disorder, lacking in particular the early onset in childhood that usually interferes with educational achievement and prohibits most public careers.

Complications. Illiteracy and Substance Use Disorders are frequent complications.

Predisposing factors. Predisposing factors are Attention Deficit Disorder and Conduct Disorder during prepuberty. The absence of parental discipline apparently increases the likelihood that Conduct Disorder will develop into Antisocial Personality Disorder. Other predisposing factors include extreme poverty, removal from the home, and growing up without parental figures of both sexes.

Age at onset. By definition the disorder begins before the age of 15. Females' first symptoms usually begin in puberty, whereas males' are usually obvious in early childhood.

Sex ratio. The disorder is much more common in males than in females.

Prevalence. Estimates of the prevalence of Antisocial Personality Disorder for American men are about 3%, and for American women, less than 1%. The disorder is more common in lower-class populations, partly because it is associated with impaired earning capacity and partly because fathers of those with the disorder frequently have the disorder themselves, and consequently their children often grow up in impoverished homes.

Familial pattern. Antisocial Personality Disorder is particularly common in the fathers of both males and females with the disorder. Studies attempting to separate genetic from environmental influences within the family suggest that both are important, since there seems to be inheritance from biological fathers separated from their offspring early in life and a social influence from adoptive fathers. Because of a tendency toward assortative mating, the children of women with Antisocial Personality Disorder who have the disorder themselves are likely to have both a mother and a father with the disorder.

Differential diagnosis. **Conduct Disorder** consists of the typical childhood signs of Antisocial Personality Disorder. Since such behavior may terminate spontaneously or evolve into other disorders such as Schizophrenia, a diagnosis of Antisocial Personality Disorder should not be made in children; it is reserved for adults (18 or over), who have had time to show the full longitudinal pattern.

Adult Antisocial Behavior, in the category Conditions Not Attributable to a Mental Disorder, should be considered when criminal or other aggressive or antisocial behavior occurs in individuals who do not meet the full criteria for Antisocial Personality Disorder and whose antisocial behavior cannot be attributed to any other mental disorder.

When **Substance Abuse** and antisocial behavior begin in childhood and continue into adult life, both Substance Use Disorder and Antisocial Personality Disorder should be diagnosed if the criteria for each disorder are met, regardless of the extent to which some of the antisocial behavior may be a consequence of the Substance Use Disorder, e.g., illegal selling of drugs, or the assaultive behavior associated with Alcohol Intoxication. When antisocial behavior in an adult is associated with a Substance Use Disorder, the diagnosis of Antisocial Personality Disorder is not made unless the childhood signs of Antisocial Personality Disorder were also present and continued without a remission of five years or more between age 15 and adult life.

Severe Mental Retardation and **Schizophrenia** preempt the diagnosis of Antisocial Personality Disorder, because at the present time there is no way to determine when antisocial behavior that occurs in an individual with severe Mental Retardation or Schizophrenia is due to these more severe disorders or to Antisocial Personality Disorder.

Manic episodes may be associated with antisocial behavior. The differential diagnosis is easily made by noting the absence of severe behavior problems in childhood and the sudden change in adult behavior.

Diagnostic criteria for Antisocial Personality Disorder

A. Current age at least 18.

B. Onset before age 15 as indicated by a history of three or more of the following before that age:

- (1) truancy (positive if it amounted to at least five days per year for at least two years, not including the last year of school)
- (2) expulsion or suspension from school for misbehavior
- (3) delinquency (arrested or referred to juvenile court because of behavior)
- (4) running away from home overnight at least twice while living in parental or parental surrogate home
- (5) persistent lying
- (6) repeated sexual intercourse in a casual relationship
- (7) repeated drunkenness or substance abuse
- (8) thefts
- (9) vandalism
- (10) school grades markedly below expectations in relation to estimated or known IQ (may have resulted in repeating a year)
- (11) chronic violations of rules at home and/or at school (other than truancy)
- (12) initiation of fights

C. At least four of the following manifestations of the disorder since age 18:

- (1) inability to sustain consistent work behavior, as indicated by any of the following: (a) too frequent job changes (e.g., three or more jobs in five years not accounted for by nature of job or economic or seasonal fluctuation), (b) significant unemployment (e.g., six months or more in five years when expected to work), (c) serious absenteeism from work (e.g., average three days or more of lateness or absence per month, (d) walking off several jobs without other jobs in sight (Note: similar behavior in an academic setting during the last few years of school may substitute for this criterion in individuals who by reason of their age or circumstances have not had an opportunity to demonstrate occupational adjustment)
- (2) lack of ability to function as a responsible parent as evidenced by one or more of the following: (a) child's malnutrition, (b) child's illness resulting from lack of minimal hygiene standards, (c) failure to obtain medical care for a seriously ill child, (d) child's dependence on neighbors or nonresident relatives for food or shelter, (e) failure to arrange for a caretaker for a child under six when parent

- is away from home, (f) repeated squandering, on personal items, of money required for household necessities
- (3) failure to accept social norms with respect to lawful behavior, as indicated by any of the following: repeated thefts, illegal occupation (pimping, prostitution, fencing, selling drugs), multiple arrests, a felony conviction
- (4) inability to maintain enduring attachment to a sexual partner as indicated by two or more divorces and/or separations (whether legally married or not), desertion of spouse, promiscuity (ten or more sexual partners within one year)
- (5) irritability and aggressiveness as indicated by repeated physical fights or assault (not required by one's job or to defend someone or oneself), including spouse or child beating
- (6) failure to honor financial obligations, as indicated by repeated defaulting on debts, failure to provide child support, failure to support other dependents on a regular basis
- (7) failure to plan ahead, or impulsivity, as indicated by traveling from place to place without a prearranged job or clear goal for the period of travel or clear idea about when the travel would terminate, or lack of a fixed address for a month or more
- (8) disregard for the truth as indicated by repeated lying, use of aliases, "conning" others for personal profit
- (9) recklessness, as indicated by driving while intoxicated or recurrent speeding

D. A pattern of continuous antisocial behavior in which the rights of others are violated, with no intervening period of at least five years without antisocial behavior between age 15 and the present time (except when the individual was bedridden or confined in a hospital or penal institution).

E. Antisocial behavior is not due to either Severe Mental Retardation, Schizophrenia or manic episodes.

301.83 Borderline Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there is instability in a variety of areas, including interpersonal behavior, mood, and self-image. No single feature is invariably present. Interpersonal relations are often intense and unstable, with marked shifts of attitude over time. Frequently there is impulsive and unpredictable behavior that is potentially physically self-damaging. Mood is often unstable, with marked shifts from a normal mood to a dysphoric mood or with inappropriate, intense anger or lack of control of anger. A profound identity disturbance may be manifested by uncertainty about several issues relating to identity, such as self-image, gender identity, or long-term goals or values. There may be problems tolerating being alone, and chronic feelings of emptiness or boredom.

Some conceptualize this condition as a level of personality organization rather than as a specific Personality Disorder. As noted below, Borderline Personality Disorder is frequently associated with other Personality Disorders.

Associated features. Frequently this disorder is accompanied by many features of other Personality Disorders such as Schizotypal, Histrionic, Narcissistic, and Antisocial Personality Disorders. In many cases more than one diagnosis is warranted. Quite often social contrariness and a generally pessimistic outlook are seen. Alternation between dependency and self-assertion is common. During periods of extreme stress transient psychotic symptoms of insufficient severity or duration to warrant an additional diagnosis may occur.

Impairment. Often there is considerable interference with social or occupational functioning.

Complications. Dysthymic Disorder and Major Depression as well as psychotic disorders such as Brief Reactive Psychosis may be complications.

Sex ratio. The disorder is more commonly diagnosed in women.

Prevalence. Borderline Personality Disorder is apparently common.

Predisposing factors and familial pattern. No information.

Differential diagnosis. In **Identity Disorder** there is a similar clinical picture and this diagnosis preempts the diagnosis of Borderline Personality Disorder if the individual is under 18. In **Cyclothymic Disorder** there is also affective instability, but in Borderline Personality Disorder there are no hypomanic periods. However, in some cases, both disorders may coexist.

Diagnostic criteria for Borderline Personality Disorder

The following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.

A. At least five of the following are required:

- (1) impulsivity or unpredictability in at least two areas that are potentially self-damaging, e.g., spending, sex, gambling, substance use, shoplifting, overeating, physically self-damaging acts
- (2) a pattern of unstable and intense interpersonal relationships, e.g., marked shifts of attitude, idealization, devaluation, manipulation (consistently using others for one's own ends)
- (3) inappropriate, intense anger or lack of control of anger, e.g., frequent displays of temper, constant anger

- (4) identity disturbance manifested by uncertainty about several issues relating to identity, such as self-image, gender identity, long-term goals or career choice, friendship patterns, values, and loyalties, e.g., "Who am I?", "I feel like I am my sister when I am good"
- (5) affective instability: marked shifts from normal mood to depression, irritability, or anxiety, usually lasting a few hours and only rarely more than a few days, with a return to normal mood
- (6) intolerance of being alone, e.g., frantic efforts to avoid being alone, depressed when alone
- (7) physically self-damaging acts, e.g., suicidal gestures, self-mutilation, recurrent accidents or physical fights
- (8) chronic feelings of emptiness or boredom

B. If under 18, does not meet the criteria for Identity Disorder.

301.82 Avoidant Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which there are hypersensitivity to potential rejection, humiliation, or shame; an unwillingness to enter into relationships unless given unusually strong guarantees of uncritical acceptance; social withdrawal in spite of a desire for affection and acceptance; and low self-esteem.

Individuals with this disorder are exquisitely sensitive to rejection, humiliation, or shame. Most people are somewhat concerned about how others assess them, but these individuals are devastated by the slightest hint of disapproval. Consequently, they withdraw from opportunities for developing close relationships because of a fearful expectation of being belittled or humiliated. They may have one or two close friends, but these relationships are contingent on unconditional approval.

Unlike individuals with Schizoid Personality Disorder, who are socially isolated but have no desire for social relations, those with Avoidant Personality Disorder yearn for affection and acceptance. They are distressed by their lack of ability to relate comfortably to others and suffer from low self-esteem.

Associated features. Depression, anxiety, and anger at oneself for failing to develop social relations are commonly present.

Impairment. Social relations are, by definition, severely restricted. Occupational functioning may be impaired, particularly if interpersonal involvement is required.

Complications. Social Phobia may be a complication of this disorder.

Predisposing factors. Avoidant Disorder of Childhood or Adolescence predisposes to the development of this disorder.

Prevalence. Avoidant Personality Disorder is apparently common.

Familial pattern. No information.

Differential diagnosis. In **Schizoid Personality Disorder** there is also social isolation, but without a desire for social involvement and with an indifference to criticism.

In **Social Phobias** humiliation is a concern, but a specific situation, such as public speaking, is avoided rather than personal relationships. However, as noted in Complications, these disorders may coexist.

In **Avoidant Disorder of Childhood or Adolescence** there is a similar clinical picture and this diagnosis preempts the diagnosis of Avoidant Personality Disorder if the individual is under 18.

Diagnostic criteria for Avoidant Personality Disorder

The following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.

- A. Hypersensitivity to rejection, e.g., apprehensively alert to signs of social derogation, interprets innocuous events as ridicule.
- B. Unwillingness to enter into relationships unless given unusually strong guarantees of uncritical acceptance.
- C. Social withdrawal, e.g., distances self from close personal attachments, engages in peripheral social and vocational roles.
- D. Desire for affection and acceptance.
- E. Low self-esteem, e.g., devalues self-achievements and is overly dismayed by personal shortcomings.
- F. If under 18, does not meet the criteria for Avoidant Disorder of Childhood or Adolescence.

301.60 Dependent Personality Disorder

The essential feature is a Personality Disorder (p. 305) in which the individual passively allows others to assume responsibility for major areas of his or her life because of a lack of self-confidence and an inability to function independently; the individual subordinates his or her own needs to those of others on whom he or she is dependent in order to avoid any possibility of having to be self-reliant.

Such individuals leave major decisions to others. For example, an adult with this disorder will typically assume a passive role and allow his or her spouse to decide where they should live, what kind of job he or she should have, and with

which neighbors they should be friendly. A child or adolescent with this disorder may allow his or her parents to decide what he or she should wear, with whom to associate, and how to spend free time.

Generally individuals with this disorder are unwilling to make demands on the people they depend on for fear of jeopardizing the relationships and being forced to rely on themselves. For example, a wife with this disorder may tolerate a physically abusive husband for fear that he will leave her.

Individuals with this disorder invariably lack self-confidence. They tend to belittle their abilities and assets. For example, an individual with this disorder may constantly refer to himself or herself as "stupid."

Associated features. Frequently another Personality Disorder is present, such as Histrionic, Schizotypal, Narcissistic, or Avoidant Personality Disorder. Anxiety and depression are common. Unless the individual has managed to secure a permanent relationship that satisfies his or her dependency needs, there is invariably preoccupation with the possibility of being abandoned.

The individual often experiences intense discomfort when alone for more than brief periods of time.

Impairment. Occupational functioning may be impaired if the nature of the job requires independence. Social relations tend to be limited to those with the few individuals on whom one is dependent.

Complications. Dysthymic Disorder and Major Depression are common complications.

Predisposing factors. Chronic physical illness may predispose to the development of this disorder in children and adolescents. Some believe that Separation Anxiety Disorder and Avoidant Disorder of Childhood or Adolescence predispose to the development of Dependent Personality Disorder.

Prevalence and sex ratio. The disorder is apparently common, and is diagnosed more frequently in women.

Familial pattern. No information.

Differential diagnosis. In **Agoraphobia**, dependent behavior is common, but the individual is more likely to *actively* insist that others assume responsibility, whereas in Dependent Personality Disorder, the individual *passively* maintains a dependent relationship.

Diagnostic criteria for Dependent Personality Disorder

The following are characteristic of the individual's current and long-term functioning, are not limited to episodes of illness, and cause either significant impairment in social or occupational functioning or subjective distress.